

Southwest Regional Medical Center · Tucson, AZ 85701

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Santos, Maria L. **MRN:** MRN-2024-004183 **DOB:** 07/12/1971 **Age:** 52 **Sex:** Female
Insurance: UnitedHealthcare Choice Plus **ID:** UHC-AZ-5512987
Admission: 03/05/2024 10:18 **Discharge:** 03/09/2024 11:30 **LOS:** 4 Days
Attending: Dr. Patricia C. Nguyen, MD (Hospitalist) **Consultant:** Dr. Arjun S. Mehta, MD (Pulmonology)

Admitting Diagnosis: Community-Acquired Pneumonia

Discharge Diagnoses

	ICD-10	Description
Principal	J13	Pneumonia due to <i>Streptococcus pneumoniae</i>
Secondary	E11.9	Type 2 Diabetes Mellitus
Secondary	I10	Essential Hypertension
Secondary	J45.20	Mild Intermittent Asthma
Secondary	Z87.891	Personal History of Tobacco Dependence

DRG: 194 — Simple Pneumonia & Pleurisy with CC **Billed:** \$18,240.00

ADMISSION HISTORY & PHYSICAL

Date: 03/05/2024 **Time:** 11:02 **Author:** Dr. Patricia C. Nguyen, MD

Chief Complaint

"Fever, chills, and bad cough for 4 days — feels like I can't get enough air."

History of Present Illness

Ms. Santos is a 52-year-old female with T2DM, HTN, and mild intermittent asthma presenting with 4 days of productive cough, fever, and progressive dyspnea. Onset March 1 with home fever 100.1°F. Cough became productive of rust-colored sputum by March 3. Completed 5-day Azithromycin Z-pack (prescribed by PCP via telehealth on March 2) without improvement. Dyspnea progressive — today unable to walk across room without stopping. Home SpO₂ 90% this morning on personal oximeter.

Past Medical History

- Type 2 Diabetes Mellitus — Metformin 1000 mg BID, Glipizide 5 mg

- 2. Essential Hypertension — Amlodipine 5 mg
- 3. Mild Intermittent Asthma — Albuterol MDI PRN
- 4. Former smoker — quit 2014, 12 pack-year history

Allergies: Sulfa (rash)

Physical Examination

Parameter	Value	Interpretation
Temperature	38.8°C (101.8°F)	Febrile
Heart Rate	94 bpm	Normal-high
Respiratory Rate	26 breaths/min	Elevated
Blood Pressure	138/86 mmHg	Mildly elevated
SpO₂ (Room Air)	90%	Below threshold
SpO ₂ after 3L NC	96%	After O ₂ applied
Weight	68 kg, BMI 26.2	

- **Respiratory:** Tachypneic. Decreased BS/dullness/crackles right base. Bronchial breath sounds RLL.
- **CV:** Regular rhythm. No murmurs. No JVD. No edema.

PSI/PORT Score: 52 → **Class III** — Hospitalization recommended (IDSA/ATS)

Assessment & Plan

- 1. **CAP — likely bacterial** (failed Azithromycin, purulent sputum, SpO₂ 90%, PSI Class III) — Blood cultures ×2 before antibiotics. Sputum Gram stain and culture. Urinary Pneumococcal + Legionella antigens. Ceftriaxone 1g IV q24h + Azithromycin 500 mg IV q24h. 3L NC.
- 2. **T2DM** — Hold Metformin (acute illness). Glucose Q6h. Sliding scale.
- 3. **HTN** — Continue Amlodipine 5 mg.

LABORATORY RESULTS

Initial Labs — 03/05/2024, 10:35

CBC

Test	Result	Reference	Flag
WBC	18.6 K/uL	4.5–11.0	↑ HIGH
Neutrophils	88%	—	↑ HIGH
Bands	6%	0–5%	↑ Elevated
Hemoglobin	13.1 g/dL	12–16	WNL

Inflammatory Markers

Test	Result	Reference	Flag
CRP	142.8 mg/L	<10	↑ MARKEDLY ELEVATED
Procalcitonin	0.84 ng/mL	<0.25	↑ Bacterial etiology suggested

ABG (Room Air) — 03/05/2024 11:15

Parameter	Value	Flag
pH	7.44	WNL
PaO ₂	62 mmHg	↓ LOW
SaO ₂	91%	↓ LOW

Urinary Antigen Tests

Test	Result
Legionella UAT	NEGATIVE
Streptococcus pneumoniae UAT	POSITIVE

Blood Cultures ×2 (03/05 — before antibiotics): No growth at 48 hours

Sputum Culture (03/05 11:00)

Field	Value
Gram Stain	Many PMNs · Gram-positive diplococci in pairs
Culture	<i>Streptococcus pneumoniae</i> — confirmed
Susceptibility	Penicillin · Ceftriaxone · Azithromycin

Follow-up Labs

Date	WBC	CRP	Trend
03/06	14.8 K/uL	98.4 mg/L	↓ Improving
03/07	10.2 K/uL	62.1 mg/L	↓ Improving
03/08	8.4 K/uL	31.8 mg/L	↓ Improving

RADIOLOGY

Chest X-Ray (PA/Lateral) — 03/05/2024 10:45 (Dr. Yolanda Kim, Radiology)

Impression: Right lower lobe lobar consolidation with air bronchograms. Moderate right pleural effusion. No contralateral infiltrate. Pattern compatible with bacterial (Streptococcal) pneumonia.

Chest X-Ray (PA) — 03/08/2024 09:00 (Dr. Yolanda Kim, Radiology)

Impression: Interval improvement in right lower lobe consolidation. Decreased pleural effusion. No new infiltrate.

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	SpO ₂	O ₂
03/05	10:18	38.8	94	26	138/86	90% RA	Room Air
03/05	11:30	38.8	94	26	138/86	96%	3L NC
03/06	06:00	38.2	88	23	132/80	96%	3L NC
03/06	12:00	37.9	86	22	130/80	97%	2L NC
03/07	06:00	37.3	80	19	126/78	97%	2L NC
03/07	18:00	37.0	76	17	122/74	98%	Room Air
03/08	06:00	36.9	74	16	120/74	98%	Room Air
03/09	09:00	36.8	72	15	118/72	98%	Room Air

PROGRESS NOTES

03/06/2024 08:30 — Dr. Patricia C. Nguyen, MD

S. pneumoniae UAT POSITIVE. Sputum: gram-positive diplococci (preliminary). CRP 98.4 ↓. Blood Cx no growth ×24h. IV Ceftriaxone + Azithromycin continuing. Pulmonology consulted. Plan transition to oral antibiotics tomorrow if stability criteria met.

03/07/2024 08:00 — Dr. Patricia C. Nguyen, MD

Stability Criteria Met: Temp 37.3°C · HR 80 · RR 19 · SpO₂ 97% on 2L NC · Tolerating PO. Transitioning to oral Amoxicillin-Clavulanate 875 mg BID + Azithromycin 500 mg daily. Weaning O₂ to room air.

03/09/2024 10:00 — Dr. Patricia C. Nguyen, MD (Discharge Note)

On oral antibiotics ×48h. SpO₂ 98% RA. Afebrile ×48h. All 6 IDSA stability criteria met. Ambulating without difficulty. **DISCHARGE TO HOME.**

DISCHARGE SUMMARY

Date: 03/09/2024 **Author:** Dr. Patricia C. Nguyen, MD **e-Signature:** Verified — 03/09/2024 10:45

Hospital Course

Ms. Santos admitted March 5 with pneumococcal pneumonia — *S. pneumoniae* confirmed by urinary antigen (POSITIVE) and sputum culture. PSI Class III. Failed outpatient Azithromycin. IV Ceftriaxone + Azithromycin with clear clinical improvement. Transitioned to oral antibiotics Day 3. O₂ weaned to RA by Day 3. Discharged Day 4 meeting all clinical stability criteria.

Discharge Medications

- Amoxicillin-Clavulanate 875 mg PO BID ×5 days (complete antibiotic course)
- Azithromycin 500 mg PO daily ×5 days
- Metformin 1000 mg PO BID (resume)
- Amlodipine 5 mg daily · Glipizide 5 mg · Albuterol MDI PRN

Condition: Good **Disposition:** Home

Follow-up: PCP in 5–7 days · Repeat CXR in 6 weeks (confirm resolution)

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-18304
Submitted	03/14/2024
Payer	UnitedHealthcare Choice Plus
DRG	194
Principal Dx	J13
Total Billed	\$18,240.00

Memorial Medical Center · Chicago, IL 60601

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Walker, James B. **MRN:** MRN-2024-005617 **DOB:** 11/03/1951 **Age:** 72 **Sex:** Male
Insurance: Medicare Fee-for-Service **ID:** 2XK8-TE7-WL44

Admission: 02/19/2024 08:40 **Discharge:** 02/25/2024 13:00 **LOS:** 6 Days
Attending: Dr. Samuel R. Thompson, MD (Cardiology) **Consultant:** Dr. Wei Zhang, MD (Nephrology)

Admitting Diagnosis: Acute Decompensated Heart Failure

Discharge Diagnoses

	ICD-10	Description
Principal	I50.23	Acute-on-Chronic Systolic Heart Failure
Secondary	N17.9	Acute Kidney Injury
Secondary	R57.0	Cardiogenic Shock
Secondary	I25.10	Coronary Artery Disease
Secondary	E11.9	Type 2 Diabetes Mellitus
Secondary	I48.19	Persistent Atrial Fibrillation
Secondary	N18.3	CKD Stage 3
Secondary	E87.1	Hyponatremia

DRG: 291 — Heart Failure & Shock with MCC **Billed:** \$31,800.00

ADMISSION HISTORY & PHYSICAL

Date: 02/19/2024 **Time:** 09:22 **Author:** Dr. Samuel R. Thompson, MD

Chief Complaint

"I woke up at 3 AM unable to breathe lying flat. I've been sleeping in my recliner 3 nights. My ankles have been getting bigger for two weeks."

History of Present Illness

Mr. Walker is a 72-year-old male with ischemic cardiomyopathy (LVEF 22%, December 2023 echo), CAD post-2-vessel CABG 2018, persistent AF, T2DM, and CKD Stage 3 presenting with 2-week worsening bilateral ankle edema, 3-day orthopnea, and paroxysmal nocturnal dyspnea. Home weight monitoring shows +4.8 kg gain (dry weight 88.0 kg; today 92.8 kg). Self-escalated Furosemide to 40 mg TID (home dose: 40 mg BID) without improvement. Marked fatigue. Inability to complete ADLs without stopping.

Past Medical History

- 1. Ischemic cardiomyopathy — LVEF 22% (December 2023)
- 2. CAD — s/p 2-vessel CABG (LIMA-LAD, SVG-OM) 2018
- 3. Persistent AF — Apixaban 5 mg BID, Digoxin 0.125 mg

- 4. T2DM — Metformin 500 mg BID, Glipizide 10 mg
- 5. CKD Stage 3 — **Baseline Creatinine 1.3 mg/dL** (January 2024)
- 6. Hypothyroidism — Levothyroxine 75 mcg

Allergies: NKDA

Physical Examination

Parameter	Value	Interpretation
Temperature	36.8°C	Afebrile
Heart Rate	98 bpm (irregular — AF)	Tachycardic
Respiratory Rate	24 breaths/min	Elevated
Blood Pressure	108/64 mmHg	Low-normal
SpO₂ (Room Air)	91%	Below normal
SpO ₂ after 2L NC	96%	After O ₂ applied
Weight	92.8 kg	+4.8 kg above dry weight (88.0 kg)

- **JVD:** +10 cm at 45° — markedly elevated
- **Cardiac:** Irregularly irregular. **S3 gallop** present. Peripheral pulses diminished.
- **Respiratory:** Bibasilar crackles to mid-lung fields bilaterally. Diminished BS at bases.
- **Extremities:** 3+ **bilateral pitting edema to mid-thigh**. Hepatomegaly. Hepatojugular reflux positive.

Assessment & Plan

1. **Acute-on-Chronic Systolic HF Exacerbation** — Admit to cardiac step-down. IV Furosemide 80 mg BID. Fluid restriction 1.5L/day. Daily standing weights. 2D Echo ordered Day 2. HOLD Carvedilol (low BP). HOLD Lisinopril (AKI concern).
2. **AKI on CKD** — Cr 1.8 vs baseline 1.3. Hold Metformin/Lisinopril. Daily creatinine. Nephrology consult. Consistent with Cardiorenal Syndrome Type 1.
3. **Dilutional Hyponatremia (Na 130)** — Manage with fluid restriction. Correct as HF treated.
4. **Persistent AF** — Continue Apixaban. Check Digoxin level.

Physician note: "Patient arrived hypotensive with BP 108/64 and low-flow state — borderline cardiogenic shock presentation."

LABORATORY RESULTS

Initial Labs — 02/19/2024

Test	Result	Reference	Flag
BNP	1,840 pg/mL	<100	↑ MARKEDLY ELEVATED
Troponin I	0.08 ng/mL	<0.04	↑ Mild (chronic HF pattern)
Hemoglobin	9.8 g/dL	13.5–17.5	↓ LOW
Sodium	130 mEq/L	136–145	↓ Hyponatremia
BUN	38 mg/dL	7–20	↑ HIGH
Creatinine	1.8 mg/dL	Baseline: 1.3	↑ AKI (+0.5, ×1.38 baseline)
eGFR	34 mL/min	Baseline: ~47	↓ Acute decline
Digoxin	1.6 ng/mL	0.5–2.0	Therapeutic

Follow-up Labs

Date	Cr	Na	BNP	Weight	SpO ₂	Notes
02/20	2.1 ↑	130	1,420 ↓	90.9 kg	95% 2L NC	Cr worsening — Cardiorenal
02/21	2.3 ↑ (peak)	131	1,010 ↓	89.6 kg	95% 2L NC	
02/22	2.0 ↓	133	724 ↓	88.8 kg	95% RA	Criteria met
02/23	1.7 ↓	135	not repeated	88.4 kg	96% RA	
02/24	1.5 ↓	136	—	88.2 kg	96% RA	Near baseline

CARDIAC IMAGING

Chest X-Ray (PA/Lateral) — 02/19/2024 (Dr. Anna Breckenridge, Radiology)

Impression: Moderate cardiomegaly. Bilateral interstitial pulmonary edema with Kerley B lines. Bilateral pleural effusions, right > left. Upper lobe vascular redistribution. Consistent with acute CHF.

2D Echocardiogram — 02/20/2024 11:30 (Dr. Samuel Thompson, MD)

- LVEF: 22% (severely reduced; worsened from 25% in December 2023)
- Severe global hypokinesis. LV dilated (LVEDD 6.8 cm).

- **Moderate Mitral Regurgitation — NEW finding** (not present on December 2023 echo)
- Grade 3 diastolic dysfunction. Estimated RVSP: 48 mmHg. No pericardial effusion.

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	MAP	SpO ₂	Wt (kg)	O ₂
02/19	08:40	36.8	98AF	24	108/64	79	91% RA	92.8	Room Air
02/19	09:30	36.8	98AF	24	108/64	79	96%	92.8	2L NC
02/20	06:00	36.8	92AF	21	114/70	85	95%	90.9	2L NC
02/21	06:00	36.7	88AF	20	116/72	87	95%	89.6	2L NC
02/22	06:00	36.7	84AF	18	118/72	87	95% RA	88.8	Room Air
02/23	06:00	36.8	82AF	17	120/74	89	96% RA	88.4	Room Air
02/24	06:00	36.7	80AF	16	118/72	87	96% RA	88.2	Room Air

Vasopressors administered: None throughout entire hospitalization.

PROGRESS NOTES

02/21/2024 13:00 — Dr. Wei Zhang, MD (Nephrology Consult)

AKI Stage 1 by KDIGO criteria — Cr 1.8 → 2.3 vs baseline 1.3 (>0.3 mg/dL within 48h; ×1.77 baseline). Consistent with Cardiorenal Syndrome Type 1. Continue diuresis cautiously. Hold Lisinopril and Metformin. No RRT indication at this time.

02/22/2024 08:30 — Dr. Samuel R. Thompson, MD

O: SpO₂ 95% RA · Weight 88.8 kg (near dry weight 88.0) · JVD resolved · Edema 1+ · BNP 724 ↓
A/P: Clinical improvement excellent. Weight near dry weight. Transitioning to oral Torsemide 20 mg daily. Resume Lisinopril 2.5 mg (half-dose). Planning discharge in 1–2 days when services are arranged.

02/23/2024 09:00 — Dr. Samuel R. Thompson, MD

Patient clinically stable. Home health services confirmed for February 25. Discharge planned for Monday. Cardiac rehab referral placed.

02/24/2024 09:00 — Dr. Samuel R. Thompson, MD

Patient stable. Weight 88.2 kg. Cr 1.5. Ready for discharge. Home health starts tomorrow.

DISCHARGE SUMMARY

Date: 02/24/2024 Author: Dr. Samuel R. Thompson, MD e-Signature: Verified — 02/24/2024 12:00

Hospital Course

Mr. Walker admitted February 19 with ADHF — 4.8 kg above dry weight, 3+ bilateral edema, orthopnea, BNP 1,840. Echo: LVEF 22% (worsened from 25%); new moderate MR identified. AKI (Cr peak 2.3) resolved with HF treatment (cardiorenal syndrome Type 1). Hyponatremia corrected. Excellent response to IV Furosemide. Transitioned to oral Torsemide.

Discharge Medications

- Torsemide 20 mg daily (replacing Furosemide — improved bioavailability)
- Carvedilol 25 mg BID (resume) · Lisinopril 2.5 mg daily (resume — increase after Cr stable)
- Spironolactone 25 mg · Apixaban 5 mg BID · Digoxin 0.125 mg
- Atorvastatin 80 mg nightly · Levothyroxine 75 mcg
- Glipizide 10 mg (resume — hold Metformin until Cr stable)

Condition: Good Disposition: Home with home health aide (3×/week)

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-29413
Submitted	03/01/2024
Payer	Medicare FFS
DRG	291
Principal Dx	I50.23
Total Billed	\$31,800.00

Premier Heart Institute · Dallas, TX 75219

INPATIENT MEDICAL RECORD — CONFIDENTIAL

Patient: Kwan, Robert T. **MRN:** MRN-2024-006829 **DOB:** 10/18/1959 **Age:** 64 **Sex:** Male
Insurance: Humana Medicare Advantage Gold **ID:** HUM-MA-77421983
Admission: 05/12/2024 14:22 **Discharge:** 05/18/2024 11:00 **LOS:** 6 Days
Attending: Dr. Victor L. Harmon, MD (Interventional Cardiology) **Consultant:** Dr. Layla Osei, MD (Gastroenterology — Day 4)

Admitting Diagnosis: Acute Chest Pain — Possible MI

Discharge Diagnoses

	ICD-10	Description
Principal	I21.09	ST Elevation MI (STEMI), other sites
Secondary	R57.0	Cardiogenic Shock
Secondary	I25.10	Coronary Artery Disease
Secondary	I48.0	Paroxysmal Atrial Fibrillation
Secondary	E11.9	Type 2 Diabetes Mellitus
Secondary	I10	Essential Hypertension
Secondary	K92.1	Melena (GI Bleed — hospital-acquired)
Secondary	D62	Acute Post-Hemorrhagic Anemia
Secondary	E78.5	Hyperlipidemia

DRG: 280 — Acute MI & STEMI with MCC **Billed:** \$78,400.00

ADMISSION HISTORY & PHYSICAL

Date: 05/12/2024 **Time:** 15:10 **Author:** Dr. Victor L. Harmon, MD

Chief Complaint

"Crushing chest pressure and sweating started 2 hours ago — radiating to my left arm."

History of Present Illness

Mr. Kwan is a 64-year-old male with CAD, T2DM, HTN, hyperlipidemia, and paroxysmal AF presenting with 2-hour substernal chest pressure (9/10), radiation to left arm and jaw, diaphoresis, and nausea. Onset during yard work. No prior MI. Equivocal stress test November 2023 (further

workup deferred by patient). No prior catheterization.

Past Medical History

- 1. CAD — non-obstructive disease on CT angiography 2022
- 2. T2DM — Metformin 1000 mg BID, Sitagliptin 100 mg
- 3. HTN — Amlodipine 10 mg, Losartan 100 mg
- 4. Hyperlipidemia — Rosuvastatin 40 mg nightly
- 5. Paroxysmal AF — Rivaroxaban 20 mg daily
- 6. GERD — Omeprazole 20 mg daily

Allergies (EMR Allergy Module): ⚠️ **ASPIRIN — GI BLEEDING**

Physical Examination

Parameter	Value	Interpretation
Temperature	37.1°C	Afebrile
Heart Rate	88 bpm	Normal
Respiratory Rate	18 breaths/min	Normal
Blood Pressure	142/88 mmHg	Elevated
SpO ₂	97% Room Air	Normal
Weight	82 kg, BMI 27.4	

- **CV:** Regular rhythm. S1/S2 present. No murmurs. No JVD. No edema.
- **Respiratory:** Clear to auscultation bilaterally.

12-Lead ECG (05/12 14:35)

Finding	Detail
Rhythm	Sinus, HR 88
ST Changes	ST depression V4–V6 (1.5–2 mm)
T-Waves	Inversion V3–V5
ST Elevation	None in any lead
LBBB	Absent
Interpretation (Dr. Harmon)	"Findings consistent with NSTEMI or demand ischemia"

Assessment & Plan

- 1. **Suspected NSTEMI** — Admit to Telemetry (Room 318). Troponin q6h ×3 sets. 12-lead ECG q8h. Aspirin 325 mg PO ×1 loading dose STAT. Heparin IV per weight-based protocol. Clopidogrel 600 mg PO ×1 loading, then 75 mg daily. Rosuvastatin 80 mg PO ×1. Urgent cardiac catheterization scheduled May 14.

LABORATORY RESULTS

Emergency Department Allergy Band Placed 14:40 — RN Jessica Okafor:
"Allergy band placed: ASPIRIN — GI BLEEDING. Physician notified."
Aspirin 325 mg administered: 15:12 (32 minutes after allergy band placed)

Troponin I (High-Sensitivity) — Serial

Date/Time	Result	ULN (52 ng/L)	Trend
05/12 14:45	2,847 ng/L	×55 ULN	↑ MARKEDLY ELEVATED
05/12 20:45	8,412 ng/L	×162 ULN	↑ Rising
05/13 02:45	12,340 ng/L	×237 ULN	↑ Peak
05/13 08:45	9,841 ng/L		↓ Declining
05/14 08:00	3,214 ng/L		↓ Declining

Heparin Monitoring

Parameter	Protocol Requirement	Actual
Bolus	80 U/kg × 82 kg = 6,560 U	5,000 U (flat dose)
Infusion	18 U/kg/hr × 82 kg = 1,476 U/hr	1,000 U/hr (flat dose)
PTT 05/12 20:00	Target 60–100 sec	42 sec — SUB-THERAPEUTIC
PTT 05/13 06:00	Target 60–100 sec	48 sec — SUB-THERAPEUTIC
Subsequent PTT	Per protocol q6h	Not ordered. No dose adjustment made.

Antiplatelet Medications in MAR

Medication	Dates Active
Clopidogrel 75 mg daily	05/12 onwards
Ticagrelor 90 mg BID	Added 05/13 onwards (both P2Y12 agents simultaneously active)

CBC — Day 4 (05/15/2024)

Test	Result	Flag
Hemoglobin	8.4 g/dL	↓ Dropped from 14.8 — acute 6.4 g/dL decrease

Patient reported melena (black tarry stool) morning of 05/15.

CARDIAC PROCEDURES

ECG Repeat — 05/13 08:00: Sinus rhythm. Persistent T-wave inversions V3–V5. **No ST elevation. No new changes from admission ECG.**

Echocardiogram — 05/13 10:00: LVEF 42% (mildly reduced). Anterior/anterolateral hypokinesis.

Cardiac Catheterization — 05/14/2024

Document	Status
Pre-procedure nursing note (05/14 09:00)	Present — patient transported to cath lab 10:15
Catheterization report	NOT PRESENT IN RECORD
Stent implant report	NOT PRESENT IN RECORD
Fluoroscopy time log	NOT PRESENT IN RECORD
Post-procedure nursing note	NOT PRESENT IN RECORD

CPT Codes Billed

Date	CPT	Description
05/14	93454	Cardiac catheterization (diagnostic)
05/14	92928	PCI / Stent placement
05/13	99291	Critical Care, first 30–74 min
05/14	99291	Critical Care, first 30–74 min

Patient location per all nursing documentation: Room 318, Telemetry Unit (nurse-to-patient ratio 1:4) throughout entire hospitalization. No CCU admission order. No CCU nursing documentation.

VITAL SIGNS

Date	Time	Temp °C	HR	RR	BP	MAP	SpO ₂	Notes
05/12	14:22	37.1	88	18	142/88	106	97% RA	ED arrival
05/12	22:00	37.0	82	16	138/84	102	98% RA	
05/13	08:30	37.0	82	16	136/82	100	98% RA	
05/14	09:00	37.0	80	16	136/84	101	98% RA	Pre-cath note: "cardioge

Date	Time	Temp °C	HR	RR	BP	MAP	SpO ₂	Notes
								nic shock"
05/15	08:00	37.1	88	17	134/82	99	97% RA	Melena noted
05/17	08:30	37.0	78	15	132/80	97	98% RA	

Vasopressors administered: None throughout entire hospitalization.

PROGRESS NOTES

05/12/2024 22:00 — Dr. Victor L. Harmon, MD

O: T 37.0°C · HR 82 · BP 138/84 · MAP 102 · SpO₂ 98% RA

A/P: "Troponin 0.04 — borderline elevated. NSTEMI diagnosis uncertain, monitoring trend."

Heparin IV continuing. Urgent cath planned May 14.

(Laboratory troponin result for same collection: 2,847 ng/L = 55× ULN)

05/13/2024 08:30 — Dr. Victor L. Harmon, MD

Troponin peak 12,340 ng/L. ECG: no ST elevation, T-wave inversions persistent. NSTEMI confirmed by troponin trend. Adding Ticagrelor 90 mg BID (in addition to Clopidogrel 75 mg already active).

05/14/2024 09:00 — Dr. Victor L. Harmon, MD

Patient in cardiogenic shock — will proceed with urgent cath. Cardiac catheterization scheduled today. Patient consented. NPO.

05/15/2024 11:30 — Dr. Layla Osei, MD (Gastroenterology Consult)

Melena and hemoglobin drop 14.8 → 8.4 g/dL. Patient on: Aspirin (administered 05/12 despite documented GI bleeding allergy), Clopidogrel, Ticagrelor, and Heparin. No allergy override documentation found. **Aspirin administration appears to have contributed to upper GI hemorrhage.**

Plan: Urgent endoscopy (May 16). 2U pRBC transfusion ordered. Hold all antiplatelets. IV Pantoprazole 40 mg q12h. NPO.

05/13/2024 14:00 — RN Mary Collins (Code Status Documentation)

Wife Susan Kwan arrived with signed POLST form (DNR/DNI, dated 03/2024, from PCP Dr. Rivera). Document photocopied. Wife stated: "Are you sure he doesn't want to be resuscitated? He was very clear about that." POLST not scanned into EMR. Chart code status remains: **Full Code**.

DISCHARGE SUMMARY

Date: 05/18/2024 **Author:** Dr. Victor L. Harmon, MD **e-Signature:** Verified — 05/18/2024 10:42

Hospital Course

Mr. Kwan was admitted May 12 with acute chest pain and was found to have an acute MI. Troponin was marginally elevated initially. He underwent cardiac catheterization on May 14 with successful PCI/stent placement. **Hospital course was otherwise uncomplicated.** He was discharged home on May 18 in stable condition.

Discharge Medications

- Clopidogrel 75 mg daily · Rosuvastatin 80 mg nightly · Amlodipine 10 mg
- Losartan 100 mg · Omeprazole 40 mg · Pantoprazole 40 mg BID ×4 weeks
- Sitagliptin 100 mg · Rivaroxaban 20 mg (resume)

Condition: Good **Disposition:** Home

CLAIM INFORMATION

Field	Value
Claim Number	CLM-2024-58291
Submitted	05/24/2024
Payer	Humana Medicare Advantage Gold
DRG	280
Principal Dx	I21.09
Total Billed	\$78,400.00